
DIAGNOSIS:

1) ACUTE GASTROENTERITIS WITH DEHYDRATION

2) URINARY TRACT INFECTION

HISTORY: C/O Multiple episodes of loose stools, 2-3 episodes of vomiting, fatigue since 3 days and fever since yesterday.

Initially she was treated at local clinic.

PAST HISTORY:K/C/O Thyroid disorder on treatment.

PHYSICAL EXAMINATION:

PR-89/min, BP-130/80 mmHg, RR-20/min, SPO2-98% at room air.

CNS-Conscious Oriented

CVS-S1S2(+)

RS-B/L NVBS(+)

PA-Soft, non tender

INVESTIGATIONS:

Reports Enclosed

COURSE IN THE HOSPITAL:

Patient presented to us with above mentioned complaints. After evaluation, she was admitted to ward. Initial investigations showed normal CBC. Serum creatinine (1.65mg/dl) done was elevated. Serum electrolytes done showed low serum sodium(128.00mmol/L). Urine routine done showed ketone bodies(+), 10-12/hpf of pus cells, 15-20/hpf of epithelial cells with presence of bacteria hence urine culture and sensitivity sent- report awaited. She was treated with IV fluids,

IV antibiotics, IV PPI's, IV antiemetics and other supportive measures. USG abdomen and pelvis done showed Grade-I fatty liver changes and mildly edematous part of ascending colon upto the hepatic flexure- could represent colitis. Repeat Serum Creatinine(1.17mg/dl) done was normal. TSH and Free T4 done were normal. Stool routine done showed 2-3/hpf of red blood cells, plenty/hpf of pus cells. Patient was adviced to stay back for further management but attenders not willing to the same, hence being discharged at request with following advice.

CONDITION AT DISCHARGE:

Hemodynamically stable

ADVICE ON DISCHARGE:

No.	Medication Name	Dosage	Frequency	Duration
1	TAB. RACIPER	40MG	1-0-0	7 DAYS (BEFORE FOOD)
2	TAB. EMESET	4MG	1-1-1	3 DAYS
3	TAB. OFLOX TZ		1-0-1	5 DAYS
4	TAB M STRONG		1-0-0	15 DAYS
5	TAB. ZEDOTT		1-1-1	3 DAYS
6	TAB. ENTRC		1-0-1	3 DAYS
7	TAB. MEFTAL SPAS		1 TAB SOS	4 TABLETS
8	TAB. LOPIRAMIDE	2MG	1-0-1	5 DAYS

FOLLOW-UP INSTRUCTIONS:

Urine culture and sensitivity sent- report awaited.

Review immediately in case of fever, loose stools, vomiting, fatigue.

Review on 09.03.2026. CBC

ER OBSERVATION CHART(1)

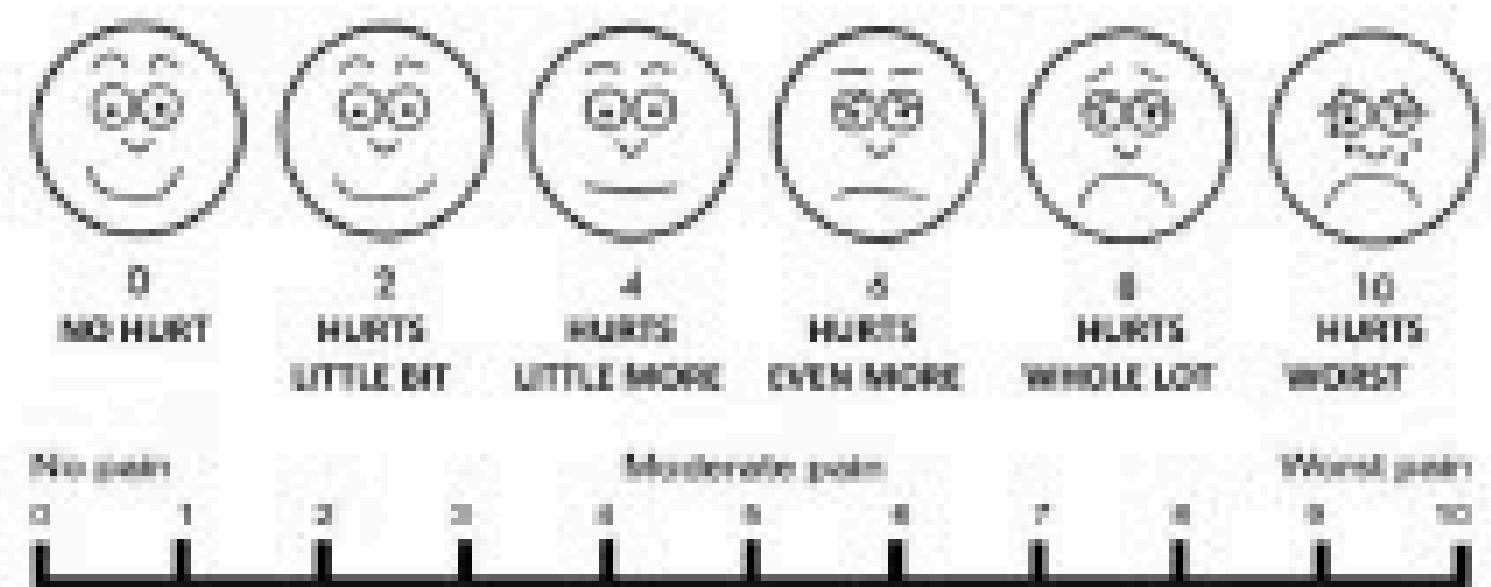
MRN:_____	Triage Category Red Yellow ☒ Green
ER ☒ OP ☐	MLC No-_____
Diagnosis: -DKA	Time of arrival ER Doctor: Time of response Time of response
	6:14pm 6:14pm

VITAL PARAMETERS

Time	6:14pm			Time	6:14pm		
Pulse	116b/m			Sao2 Oxygen			
BP	87/50			RR	22b/m		
Temp	98°F			Urine output (ml/hr)			
SaO2 on air	96%			Blood Glucose	443mg/dl		
GCS	15/5			Pain score	4/10		

Procedures _____ Pain scale 4/10

-> IV cannulization done in left hand 20g



Note: If score is > 2 please speak to consultant

Investigations: (Lab,Radiology,Pathology,others)	Time (patient /sample) sent	Time (patient / report) received	Referred To: Specialty Specialty Specialty Specialty Dr. Consultant	ie
CBL, creat, SE				
ABG, urine	Report	Report		
ECG	Sent to lab	Received		
			other orders:	